Word count: 1,443

Healing, Housing, and Hope: Medical Respite Care at Boston Health Care for the Homeless Program

Program Brief

Introduction

Unhoused individuals recovering from acute illness, injury, or surgery often face prolonged suffering and poor outcomes due to a lack of safe recovery spaces. The Medical Respite Care Team at Boston Health Care for the Homeless Program (BHCHP) addresses this critical gap through integrated, trauma-informed care tailored to the complex needs of this population. This brief aims to provide an overview of BHCHP's respite programming, its public health rationale, and our policy context. We review the scope of homelessness in Boston, summarize national and local policies affecting medical respite, and assess current program strengths and challenges. Finally, we outline forward-looking priorities in clinical care and advocacy based on evidence-informed practices and organizational experience.

Homelessness in Boston and the Need for Medical Respite Care

On the night of January 29, 2025, Boston counted 5,506 individuals experiencing homelessness, which was a 4.3% decrease from the previous year (City of Boston, 2025). Among them, 2,122 were single adults, and 3,384 were people in families, the latter representing a 9% year-over-year reduction. The number of adult individuals in emergency shelters increased from 1,544 to 1,648, attributed to an expansion in shelter capacity to reduce overcrowding.

Despite reductions in family homelessness and unsheltered counts, the growing number of medically complex individuals in shelters reflects an ongoing public health challenge. Veterans, unaccompanied youth, and domestic violence survivors also remain key subpopulations, though veteran homelessness alone has declined 55.2% since 2011, and domestic violence-related homelessness dropped by 55% in the past year. Boston's substantial investment in housing exits, 2,838 people transitioned to permanent housing in 2024, shows that coordinated housing, health care, and supportive services can meaningfully reduce homelessness. However, recovery from illness or surgery while unhoused remains dangerous, leading to elevated hospital readmissions and poor long-term outcomes.

Unhoused individuals are hospitalized up to five times more often than housed people, and post-discharge complications are common due to unsafe or unsanitary living conditions (Kertesz et al., 2009). Medical respite care, a short-term, residential model offering clinical and social services for unhoused individuals, fills this critical gap, improving recovery and reducing readmissions (Doran et al., 2013). The growing complexity of patient needs, particularly in the context of rising fentanyl use and aging among the homeless population, makes access to such care more essential than ever.

Policy Landscape: Massachusetts and US Context

While national recognition of medical respite care is growing, systemic support remains uneven. As of 2025, the Centers for Medicare and Medicaid Services (CMS) allow, but do not require, states to cover medical respite services under Medicaid waivers. Massachusetts has yet to fully integrate respite care into its Medicaid program, relying heavily on grants and hospital partnerships for funding (MassHealth, 2024). Recent federal guidance encourages value-based care models and social determinants of health investments, including medical respite care, under Accountable Care Organizations and Medicaid

Word count: 1,443

Managed Care (CMS, 2023). Massachusetts' 1115 waiver renewal offers a promising policy lever to formalize and scale medical respite reimbursement, but implementation remains in early stages.

At the local level, the Boston Public Health Commission and state legislators have expressed support for expanding post-acute care for unhoused individuals. However, without sustained funding streams, many programs, including BHCHP's, remain vulnerable to financial volatility.

BHCHP's Mission and Respite Services

Founded in 1985, BHCHP delivers comprehensive, dignified care to Boston's unhoused population through street outreach, shelter-based clinics, behavioral health, addiction medicine, and respite care. Our guiding mission is to provide "equitable, high-quality health care that meets people where they are." BHCHP annually serves nearly 10,000 patients through an interdisciplinary team of clinicians, case managers, recovery specialists, and community health workers.

BHCHP is governed by a Board of Directors that includes clinicians, public health experts, and individuals with lived experience of homelessness. The organization is led by president Dr. James O'Connell, a nationally recognized leader in street medicine and a founding member of the program. Funding for BHCHP's services comes from a mix of Medicaid reimbursements, hospital partnerships, government grants, and private philanthropy.

The Medical Respite Care Team operates out of the Barbara McInnis House, a 104-bed facility co-located with BHCHP's central office. The program accepts referrals from hospitals, shelters, and outreach teams and provides 24/7 nursing care, primary and specialty medical services, medication-assisted treatment, case management, behavioral health support, and connections to permanent housing. On average, patients stay 28 days, with individualized plans developed to optimize recovery and transition support.

BHCHP's partners include Mass General Brigham, Boston Medical Center, Pine Street Inn, and the Boston Public Health Commission. While collaboration remains strong, siloed funding streams and shifting policy priorities have required adaptive and proactive strategies to maintain program stability.

Evidence Review: Effectiveness of Medical Respite Care

Evidence from peer-reviewed research supports the value of medical respite care as a transitional and transformative model for individuals experiencing homelessness. Findings across diverse methodologies demonstrate consistent outcomes across three core themes: reductions in hospital utilization, facilitation of stable transitions, and the value of trauma-informed care. For more information, refer to table 1.

Reducing Readmissions and Acute Care Utilization: Hospital readmissions are a costly and common challenge for unhoused patients. Doran et al. (2013) found a striking 50.8% 30-day readmission rate among homeless individuals, with more than half returning within one week of discharge, an indicator of poor discharge continuity and unsafe living environments. These outcomes point to the critical need for medical respite care as a structured, post-acute alternative to discharge to the street or shelter.

Bridging the Gap to Housing and Recovery: Case study data from McCarthy and Waugh (2021) described the National Health Foundation's medical respite program, which achieved a notably low 9% readmission rate among participants, substantially below national norms. In addition, only 23% of patients returned to the emergency department, and many transitioned into stable housing with care

continuity. These findings reinforce that medical respite functions as both a health intervention and a social determinant of housing stability.

Trauma-Informed, Dignified Care Models: Qualitative studies further highlight how trauma-informed and relational care approaches enhance patient dignity and outcomes. Murphy Buc (2023) identified the emotional and psychological shifts patients experience in hospice/palliative respite settings through a grounded theory study. Participants described care as a process of “recreating family” after cumulative loss and trauma. Bowie et al. (2018) similarly found that participatory, community-led care planning, such as through the Vulnerability Index, encouraged greater trust and responsiveness in service delivery. These insights support integrating patient voice, cultural competence, and relational continuity into respite models.

Word count: 1,443

Table 1: Literature Review

Author, Year, Citation	Study Objective	Key Study Methods	Key Findings	Study Design Limitations
Hadenfeldt C, Todd MJ, Hamzhie C. *Medical respite post-hospitalization for adults experiencing homelessness. Nursing*. 2023;53(3):47-52. https://pmc.ncbi.nlm.nih.gov/articles/PMC9946160/#sec1	To assess care gaps for unhoused adults after hospital discharge and explore medical respite as a solution	• Adults experiencing homelessness, no sample size specified • Community-based qualitative assessment • Omaha, Nebraska	• Discharge to shelters or streets is unsafe • Medical respite may reduce readmissions and improve recovery • Community supports exist but are fragmented	• No quantitative data • Sample size not reported • Results not generalizable beyond local context
Bowie et al., (2018). *Using the Vulnerability Index® to assess the health needs of a homeless community. Journal of Community Health Nursing, 35*(4), 189–195. https://doi.org/10.1080/07370016.2018.1516422	Assess health needs of a homeless community using the vulnerability index (VI)	• Cross-sectional survey using VI • 47 homeless adults in Seattle • Community-based participatory research	• High rates of chronic illness • VI effective for lay-administered health needs assessment • Supports medical respite care development	• Small sample size • Data from one community • Descriptive, no causal inference
Doran et al., (2013). *The revolving hospital door: Hospital readmissions among patients who are homeless. Medical Care, 51*(9), 767–773. https://doi.org/10.1097/MLR.0b013e31829fafbb	Assess 30-day hospital readmission rates among homeless individuals	• Retrospective chart review • 113 homeless patients • 266 hospitalizations at a single urban hospital in New Haven, CT	• 50.87 30-day readmission rate • 70.3% had an acute care use • 53.9% readmitted within one week	• Single site-study • No comparison with housed patients • Only uses emergency room from one hospital
Murphy Buc, H. L. (2023). *Ain't never been cared for: A grounded theory of receiving hospice and palliative care for homeless and vulnerably housed individuals* (Doctoral dissertation, The Catholic University of America). JSTOR. https://www.jstor.org/stable/community.38760587	Explore how homeless/vulnerably housed individuals experience hospice and palliative care	• Qualitative constructivist grounded theory • 9 participants • Interviews • Analyzed patient narratives	• 3-phase process: trauma, loss of health/home, receiving care & recreating family • Emphasizes trauma-informed care	• Small sample size • Single population focus • Lacks quantitative outcome measures
McCarthy, D., & Waugh, L. (2021, August). *How a medical respite care program offers a pathway to health and housing for people experiencing homelessness*. The Commonwealth Fund. https://www.commonwealthfund.org/publications/case-study/2021/aug/medical-respite-care-pathway-health-housing	Describes the National Health Foundation's medical respite care model and its outcomes for homeless patients	• Case study of NHF in CA • Over 60 hospitals referred patients • 117 beds across 4 sites • Primarily qualitative and descriptive	• Among 64 patients, readmission rate was 9% • ED revisit rate was 23% • Stays supported housing transitions	• Case study design • Lacks control group or longitudinal data • Outcomes are not generalizable

Word count: 1,443

Strengths, Limitations, and Program Challenges

BHCHP's respite care model is among the most established in the US, offering trauma-informed, interdisciplinary care in a facility uniquely tailored to the needs of unhoused individuals. Key strengths include seamless integration with local hospitals, allowing for efficient referral pathways and coordinated care. The program's culturally competent workforce fosters trust and long-term engagement with patients who have historically faced barriers to care. In addition, BHCHP has demonstrated strong clinical outcomes, including reductions in readmission rates and improvements in care continuity and housing transitions.

Despite these strengths, BHCHP's respite program continues to face significant challenges. Demand consistently exceeds the 104-bed capacity, resulting in waitlists, delayed discharges, and missed opportunities for timely interventions. Behavioral health needs have intensified, often outpacing on-site resources. Financial sustainability remains fragile in the absence of secure Medicaid reimbursement. Lastly, workforce burnout and staffing shortages are growing concerns as acuity levels rise. These limitations directly inform the program's future priorities in policy advocacy, service enhancement, and workforce investment.

Actionable Priorities

Building on its leadership in medical respite care and informed by internal assessment and national evidence, BHCHP has identified three interrelated priorities to guide its advocacy and programmatic development over the next 12-18 months:

1. Policy Advocacy for Medicaid Reimbursement: Partner with MassHealth and advocacy coalitions to secure inclusion of medical respite care in the state's Medicaid benefit package under the 1115 waiver. This is essential to stabilize funding and support program expansion.
2. Expand Behavioral Health Capacity: Pilot a multidisciplinary behavioral health team, including a psychiatric nurse practitioner, a licensed clinical social worker, and peer recovery coaches with lived experience. This model will prioritize trauma-informed care, MAT access, de-escalation training, and continuity with community providers.
3. Strengthen Data and Evaluation Infrastructure: Launch a longitudinal outcome tracking system, in collaboration with academic institutions to collect data on housing stability, readmission rates, emergency utilization, and patient-reported outcomes six to twelve months post-discharge.

These priorities are designed to ensure that BHCHP continues to lead the field in person-centered, evidence-based medical respite care while expanding its reach, deepening its services, and shaping the future of healthcare for people experiencing homelessness.

Word count: 1,443

References

Boston Health Care for the Homeless Program. (n.d.). *Medical respite*. https://www.bhchp.org/services/medical-respite/

Bowie et al., (2018). *Using the Vulnerability Index® to assess the health needs of a homeless community. Journal of Community Health Nursing, 35*(4), 189–195. https://doi.org/10.1080/07370016.2018.1516422

Centers for Medicare & Medicaid Services. (2023). *State Medicaid director letter: Opportunities to address social determinants of health*. https://www.medicaid.gov

City of Boston. (2025). *Annual Homeless Census Report*. https://www.boston.gov/departments/housing/annual-homeless-census

Doran et al., (2013). *The revolving hospital door: Hospital readmissions among patients who are homeless. Medical Care, 51*(9), 767–773. https://doi.org/10.1097/MLR.0b013e31829fafbb

Hadenfeldt C, Todd MJ, Hamzhie C. *Medical respite post-hospitalization for adults experiencing homelessness. Nursing*. 2023;53(3):47-52. https://pmc.ncbi.nlm.nih.gov/articles/PMC9946160/#sec1

Kertesz, S. G., Posner, M. A., O'Connell, J. J., Swain, S., Mullins, A. N., Shwartz, M., & Ash, A. S. (2009). Post-hospital medical respite care and hospital readmission of homeless persons. *Journal of prevention & intervention in the community*, *37*(2), 129–142. https://doi.org/10.1080/10852350902735734

MassHealth. (2024). *Massachusetts Medicaid 1115 Waiver Renewal Summary*. https://www.mass.gov/info-details/masshealth-section-1115-demonstration-waiver

McCarthy, D., & Waugh, L. (2021, August). *How a medical respite care program offers a pathway to health and housing for people experiencing homelessness*. The Commonwealth Fund. https://www.commonwealthfund.org/publications/case-study/2021/aug/medical-respite-care-pathway-health-housing

Murphy Buc, H. L. (2023). *Ain't never been cared for: A grounded theory of receiving hospice and palliative care for homeless and vulnerably housed individuals* (Doctoral dissertation, The Catholic University of America). JSTOR. https://www.jstor.org/stable/community.38760587